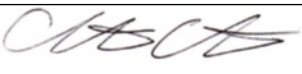


Guideline Title: Propofol (Diprivan)

Guideline Number: GUID-3203-G007

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** This policy provides a guideline for the regulation and management of Propofol (Diprivan) during Air Care Team transports.
- II. **DEFINITIONS:**
 - a. MCP (Medical Command Physician) – accepting physician of record or Air Care Medical Director
 - b. Pediatric- less than 16 years old
 - c. SBP- Systolic Blood Pressure
- III. **DEPARTMENT GUIDELINE:**
 - a. Propofol infusions can only be administered by trained RNs to patients that are intubated and have continuous EKG, blood pressure monitoring, ETCO2 monitoring, and pulse oximetry in place.
 - b. If initiating infusion
 - i. **Propofol 10 mcg/kg/min IV, titrate by 10 mcg/kg/min q5 min** to achieve a Sedation-Agitation Scale value of 2-3 (See attached chart below). Maximum infusion rate 5-50 mcg/kg/min.
 1. Any rate higher than 50 mcg/kg/min must be approved by an MCP
 - c. If continuing an existing infusion (for example, inter-facility transfer)
 - i. Continue infusion at encountered rate (even if it is above 50 mcg/kg/min) as long as patient is sedated and SBP > 90 mmHg
 - ii. For adult patients, the rate may be titrated up to 50 mcg/kg/min as needed to achieve Sedation-Agitation Scale value of 2-3.
 - iii. *For pediatric patients, Propofol doses cannot be increased from the encountered rate without an MCP order.* Pediatric doses encountered may be 100-150 mcg/kg/min.
 - d. If SBP < 90 mmHg, reduce drip slowly allowing blood pressure to stabilize prior to readjusting again.
 - e. If patient remains hypotensive, despite multiple decreases in drip rate, discontinue Propofol drip and notify referring physician of continued hypotension. Attempt to maintain needed sedation by administering other sedation medications per GUID3203-G006 – *Analgesia and Sedation Management*. If blood pressure dose not increase with a decrease in rate, assess for other causes of hypotension and treat per GUID3203-M001 – *Hypotension*.

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- f. If patient becomes agitated and SBP > 90 mmHg, titrate by 10mcg/kg/min increments, up to 50 mcg/kg/min, monitoring BP after each adjustment.
- g. If adult or pediatric pt remains agitated on 50 mcg/kg/min and SBP > 90 mmHg, administer **Fentanyl 0.5-2 mcg/kg slow IV/IO**, may repeat every 10 minutes
- h. Consider administering **Rocuronium 0.6-1 mg/kg IV** to facilitate oxygenation and ventilation in appropriately sedated (SAS ≤ 3) patients with increased airway pressures or poor ventilator tolerance.
- i. Utilize referring physician or MCP for guidance especially in the care of pediatric patients.

Sedation-Agitation Scale (SAS)

Score	Level of sedation-agitation	Response
7	Dangerous agitation	Pulling at endotracheal tube, thrashing, climbing over bed rails.
6	Very agitated	Does not calm, requires restraints, and bites endotracheal tube.
5	Agitated	Attempts to sit up but calms to verbal instructions.
4	Calm and cooperative	Obeys commands.
3	Sedated	Difficult to rouse, obeys simple commands.
2	Very sedated	Rouses to stimuli. Does not obey commands.
1	Unarousable	Minimal or no response to noxious stimuli.

IV. DOCUMENTATION:

- a. EMS Charts patient documentation software
- b. Physicians Order sheet

V. REFERENCES:

- a. Propofol Infusion Protocol for Neurotrauma patients >16 years of age Form #5872-100883
- b. Orlando Health Continuous Infusion Standard Dilutions and Dosing Guidelines